



Uttarakhand Pharma, Healthcare & AYUSH Conclave and Samman 2026

Concept & Categories Document

Innovating Healthcare: Bridging Gaps through Technology, Research and Collaboration

24 June 2026 · Dehradun, Uttarakhand

PHD Chamber of Commerce and Industry
Healthcare, Education & Research Committee

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1. Executive Summary

The **Uttarakhand Pharma, Healthcare & AYUSH Conclave and Samman 2026** is the second annual edition of a recognition platform created by the PHD Chamber of Commerce and Industry (PHDCCI) to identify, document, and celebrate excellence across the healthcare, AYUSH, and life-sciences manufacturing ecosystem of the Himalayan region. It will be held on 24 June 2026 in Dehradun, under the theme “Innovating Healthcare: Bridging Gaps through Technology, Research and Collaboration.”

This document sets out the concept, rationale, and full category architecture of the 2026 edition. It explains why the programme exists, why Uttarakhand is the natural home for it, how the recognition framework has been designed around what an expert jury can distinctly and credibly evaluate, and what nominees must submit. It is intended as the single reference for the organising team, the jury, the web and operations teams, and prospective nominees.

What the 2026 edition offers

- **Structure:** 59 recognition categories organised across five competitive tracks (A–E), one Cross-Track section open to all participants, and a Distinguished Achievers’ Citations tier — a consolidation from roughly 76 loosely defined categories in 2025 down to a coherent, defensible framework.
- **Design principle:** Every Samman is defined by a distinct evaluation question, not by stakeholder demand. Common themes — sustainability, community, innovation — are consolidated so the same idea is not judged four times by four different standards.
- **Fairness by design:** Size-band architecture in the manufacturing and nutraceutical tracks ensures emerging MSMEs never compete head-to-head with industry leaders. Public-sector and private-sector nominees are evaluated in separate peer groups in the Cross-Track section.
- **New for 2026:** A dedicated Track E for FSSAI-licensed Nutraceutical manufacturers, reflecting the fastest-growing segment of the wellness economy.
- **Transparent eligibility:** A uniform set of mandatory documents per category — GST returns to establish a functioning business, a track-relevant operating or manufacturing licence, and key team-member details — is published on each category card before a nominee applies.
- **A distinguished jury:** A panel led by a serving Vice-Chancellor and scientist, a former Drugs Controller General of India, a former State Drug Controller, and the UN Global Compact Network India, anchoring credibility and impartiality.

The central idea is simple and is carried through every page that follows: the Samman is a **discovery of excellence that goes beyond compliance**. Regulatory licences confirm that an organisation is permitted to operate; the Samman looks for what it has done above and beyond that baseline — the innovation, the outcomes, the community impact, and the institution-building that a licence can never capture.

2. The Current State of Healthcare in India

India's healthcare sector is among the largest and fastest-growing in the world. It was valued at approximately US\$372 billion in 2023 and is projected to reach US\$638 billion by 2025, growing at a compound rate of 17.5–22.5%.¹ Health spending accounted for 3.3% of GDP in 2022 and is expected to rise towards 5% by 2030 as public investment, private capital, and insurance coverage all expand together.²

The public financing picture has changed structurally. Total Health Expenditure in FY22 stood at ₹9,04,461 crore, about 3.8% of GDP. Crucially, the government's share of total health expenditure rose from 29% in FY15 to 48% in FY22, while out-of-pocket expenditure — long the weakest point of the Indian system — has been declining steadily, driven by Ayushman Bharat, the expansion of Ayushman Arogya Mandirs, and strengthened primary care.³

On capacity, India had over 13.86 lakh registered allopathic doctors and 7.51 lakh AYUSH practitioners as of April 2025, with a doctor-to-population ratio of about 1:811, and a healthcare workforce that has already crossed six million.⁴ Digital health and telemedicine, AI-assisted diagnostics, the Ayushman Bharat Digital Mission, and a wave of health-tech start-ups are reshaping how care is delivered, especially for the roughly 70% of the population in rural areas.

The challenges that persist

Alongside this growth, well-documented structural challenges remain, and they are sharpest in North India and the hill states:

- **A skewed urban–rural divide**, with infrastructure and specialists concentrated in cities while rural and hilly districts struggle to attract and retain doctors.
- **Rising non-communicable diseases** — diabetes, hypertension, cardiac disease, cancer — layered onto continuing maternal, child, and communicable-disease burdens.
- **Workforce shortages** across nurses, paramedics, radiologists, and lab technicians, and uneven quality of public health infrastructure.
- **Weak documentation and benchmarking.** The 2025 jury observed directly that the quality of documenting work and achievements across the sector was weak, that submissions often lacked hard data, that sustainability was largely compliance-driven, and that brand-building was almost non-existent.⁵

That last point is decisive for this programme. The gap is not only in delivery — it is in **recognition, evidence, and storytelling**. Excellent work is being done across the region, but it is poorly documented, rarely benchmarked, and seldom celebrated in a way that builds reputation or attracts investment. A rigorously designed recognition platform addresses exactly this gap.

¹India Brand Equity Foundation (IBEF), Healthcare Industry in India, 2025 — sector valued at US\$372 billion (2023), projected US\$638 billion by 2025; health spending 3.3% of GDP (2022), projected 5% by 2030.

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³Economic Survey 2024–25, Government of India (PIB) — Total Health Expenditure (FY22) ₹9,04,461 crore (3.8% of GDP); government share of THE rose from 29% (FY15) to 48% (FY22); out-of-pocket expenditure declining.

⁴IBEF, 2025 — 13,86,150 registered allopathic doctors and 7,51,768 AYUSH practitioners as of 1 April 2025; doctor-to-population ratio 1:811; healthcare workforce exceeding 6 million (CY24).

⁵PHDCCI Uttarakhand Pharma & Healthcare Conclave and Samman 2025 — Event Book, Jury Annotations (UPHCS 2025).

3. Uttarakhand: A Destination for Services, AYUSH Healing, and Manufacturing

Uttarakhand occupies a rare position in the Indian healthcare landscape: it is simultaneously a centre of modern clinical care, the spiritual and geographic home of AYUSH and wellness, and an established manufacturing base for both pharmaceuticals and AYUSH products. Few states combine all three. This convergence is the foundation on which the Samman is built.

3.1 Healthcare services and medical value travel

Dehradun, Haridwar, Rishikesh, and Haldwani have emerged as hubs for corporate and multi-specialty hospitals, with strong tertiary and super-specialty capacity and growing diagnostic networks. The state's connectivity through Dehradun and Delhi, its climate, and its landscape make it a natural destination for medical value travel and recovery. India already attracts around two million medical-value-travel patients a year, and Uttarakhand is well placed to capture a growing share.⁶

3.2 AYUSH healing and the wellness economy

Uttarakhand is, in the public imagination and in practice, a wellness destination. Haridwar and Rishikesh draw patrons from across the world for yoga, naturopathy, panchakarma, and Ayurvedic care. This sits inside a national AYUSH market valued at about US\$43.3 billion in 2024 and projected to reach US\$200 billion by 2030, of which manufacturing accounts for roughly US\$24 billion and services roughly US\$26 billion.⁷ India's wellness-tourism market alone was estimated at US\$19.43 billion in 2024 and is projected to reach US\$26.55 billion by 2029.⁸

The opportunity for Uttarakhand is to convert its natural and cultural advantage into accredited, well-documented, investment-ready wellness infrastructure — integrated wellness centres, Ayurveda resorts, and concierge medical services — rather than leaving it as informal, unbranded activity.

3.3 Manufacturing — pharmaceuticals and AYUSH

The Haridwar–Roorkee–Dehradun belt is one of North India's established manufacturing corridors for pharmaceuticals, AYUSH products, medical devices, and nutraceuticals.⁹ The 2025 edition itself recognised manufacturers spanning product innovation, quality and compliance, in-house testing, smart packaging, R&D, and workforce development — demonstrating both the depth of the base and the breadth of activity already present in the state.¹⁰

Yet the 2025 jury also noted that a majority of these manufacturers operate as contract manufacturers or component suppliers for larger brands, limiting their visibility and value capture.¹¹ The strategic case

⁶IBEF, Wellness & Medical Value Travel, 2024 — wellness tourism market estimated US\$19.43 billion (2024), projected US\$26.55 billion by 2029; India attracts ~2 million medical-value-travel patients annually.

⁷IBEF & Invest India, AYUSH Sector, 2024–25 — Indian AYUSH market US\$43.3 billion (2024), projected US\$200 billion by 2030; manufacturing segment ~US\$24 billion and services ~US\$26 billion in 2024.

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⁹Imarc / Mordor Intelligence, India AYUSH & Alternative Medicine Market, 2024–25 — Haridwar and the wider Uttarakhand belt identified among India's established AYUSH and pharmaceutical manufacturing hubs in North India.

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¹¹

for the Samman is therefore not merely to applaud the region, but to help its enterprises move from anonymous capacity to recognised, benchmarked, brand-building leaders — exactly the transition the state needs to make.

4. The Need and Concept of the Samman

PHDCCI launched the UPHACS to create an annual platform that engages stakeholders, shares best practices, generates a longitudinal dataset on the sector, and honours excellence. The 2026 edition refines that purpose around a single, disciplined idea.

4.1 Discovery of excellence — beyond compliance, with a vision

A regulatory licence establishes only that an organisation is **permitted to operate** — that it meets the minimum standard required by law. The Samman begins where compliance ends. It is a **discovery of excellence**: a structured search for the institutions, practitioners, and enterprises that have gone measurably beyond their statutory baseline to deliver innovation, superior outcomes, ethical leadership, community impact, and lasting institution-building.

In practice, this means the recognition framework is built to reward performance that a licence can never capture: the hospital that redesigned a clinical pathway and can prove better outcomes; the AYUSH manufacturer whose new formulation has documented health-benefit evidence; the MSME that built an in-house testing laboratory it was not obliged to build; the frontline ASHA worker whose outreach changed immunisation numbers in her block; the company whose CSR programme produced measurable community health gains.

4.2 Why a new, consolidated design was necessary

The 2025 edition carried roughly 76 categories. The root problem was that categories had been created reactively to accommodate stakeholders rather than designed around what a jury can distinctly evaluate. The same themes — innovation, sustainability, community outreach, research — appeared separately inside every track, so identical ideas were judged four times against four different definitions. Many categories attracted only one or two nominations and could not be judged competitively at all.

The 2026 design corrects this by consolidating around **evaluation purpose**, not stakeholder pressure: 59 well-defined categories, each anchored to a clear question the jury can answer with evidence; common themes pulled into a single Cross-Track section; and size-bands so enterprises of different scale compete fairly. Familiar 2025 names are retained wherever the sector recognised them, with their meaning expanded to absorb related ideas.

4.3 The fee-first operating principle

The competitive tracks operate on a fee-first basis. A nomination is confirmed, and the detailed nomination form shared, only once the registration for the chosen category is complete. This keeps the process orderly for nominees and the evaluation team alike, ensures every submission that reaches the jury is a serious one, and lets the team explain every category from a single, consistent script. The schedule of category-wise registration amounts is maintained and published separately on the official programme website and does not form part of this document.

Two sets of recognitions stand outside the fee-first model and are **not open for subscription**:

- **Track A — Public Health.** Government health facilities, departments, and frontline workers carry no nomination fee. They are recognised on the basis of established government quality frameworks — Kayakalp and NQAS — and departmental recommendation, rather than through a paid nomination.
- **Distinguished Achievers' Citations.** These citations are conferred purely at the prerogative of the jury. There is no application, no self-nomination, and no fee; recipients are identified by the jury from the competitive nominee pool, or by Letter of Recommendation and Detailed Profile for the Lifetime Achievement and Pioneer citations.

5. About PHDCCI

The PHD Chamber of Commerce and Industry (PHDCCI) is one of India's premier apex industry bodies, established in 1905 and operating under the motto "Voice of Industry & Trade." For over a century it has served as a proactive, dynamic, multi-state chamber working at the intersection of industry, government, and society, with a strong presence across North India and a national footprint.

PHDCCI works through a structure of expert committees spanning the major sectors of the economy. The Healthcare Samman is convened under its Healthcare, Education & Research vertical, which brings together hospitals, manufacturers, AYUSH institutions, regulators, academia, and development partners. The Chamber's convening power — its ability to bring a former drug regulator, a serving Vice-Chancellor, a UN body, and state government leadership to the same table — is precisely what makes a credible, independent recognition programme possible.

The 2026 Conclave is being organised with the participation of the Government of Uttarakhand, with **Shri Subodh Uniyal, Hon'ble Cabinet Minister, Government of Uttarakhand** invited as Chief Guest (confirmation awaited). PHDCCI's regional office for the programme is based in Dehradun.

Founded	1905 — one of India's oldest and largest apex chambers of commerce
Identity	Voice of Industry & Trade — a non-profit, non-government, multi-state industry body
Programme owner	Healthcare, Education & Research Committee, PHDCCI
2026 edition	2nd Annual Uttarakhand Pharma Healthcare & AYUSH Conclave and Samman — 24 June 2026, Dehradun

6. Our Jury and Their Profiles

The credibility of any recognition rests on the standing and independence of those who confer it. The 2026 Samman is guided by a distinguished panel drawn from drug regulation, science and academia, and global sustainability governance. The names and designations below are as published on the official programme website, phdhpcs.com; the profiles draw on publicly available sources.

Dr. (Prof.) Rajendra Dobhal

Vice-Chancellor, Swami Rama Himalayan University · Former Director General, UCOST

Dr. Rajendra Dobhal is a widely travelled scientist and academic leader who assumed charge as Vice-Chancellor of Swami Rama Himalayan University in 2023. Previously Director General of the Uttarakhand State Council for Science and Technology (UCOST), he is credited with shaping the science-and-technology architecture of Uttarakhand — helping establish the Vigyan Dham Science City, regional science centres, a patent information centre, climate-change and forest-livelihood centres, and multiple technology resource centres.

He has guided over 425 research and development projects and authored numerous technical reports, books, and research papers, and holds honours including Uttarakhand Ratna and Uttarakhand Gaurav. He chairs the National Academy of Sciences India (NASI), Uttarakhand Chapter. His standing in research, higher education, and Himalayan science gives the jury deep authority on the innovation, research, and institutional-excellence categories.

Dr. G. N. Singh

Advisor to the Hon'ble Chief Minister of Uttar Pradesh · Former Drugs Controller General of India (DCGI)

Dr. G. N. Singh is one of India's most senior figures in pharmaceutical regulation. He served as the Drugs Controller General of India (DCGI), the head of the Central Drugs Standard Control Organisation (CDSCO), from 2012, and earlier as Secretary-cum-Scientific Director of the Indian Pharmacopoeia Commission. A pharmacist by training (B.Pharm, M.Pharm and Ph.D), he has spent his career setting and enforcing the national standards for the manufacture, quality, import, and distribution of drugs in India.

He currently serves as Advisor to the Hon'ble Chief Minister of Uttar Pradesh, where he champions the state's positioning as a healthcare and medical-device hub and works on innovation, incubation, and regulatory collaboration with bodies such as CDSCO and BIRAC. His presence anchors the jury's authority on questions of drug quality, manufacturing standards, and regulatory excellence.

Dr. Narender Ahooja

Former State Drug Controller, Haryana

Dr. Narender Ahooja is a senior drug-regulation administrator who served as the State Drug Controller of Haryana, heading the state's Food and Drug Administration. The department, constituted as an independent regulator, enforces the Drugs and Cosmetics Act and the Food Safety and Standards Act — ensuring the supply of quality, affordable medicines and food, acting against spurious and sub-standard drugs, and overseeing manufacturing licensing.

His regulatory experience across drug quality assurance, enforcement, and manufacturing oversight brings practical, on-the-ground expertise to the evaluation of the manufacturing and compliance tracks, where the line between mere licensing and genuine quality excellence must be drawn with care.

Dr. Somnath Singh

Deputy Director, UN Global Compact Network India (UN GCNI)

Dr. Somnath Singh is a development leader and academic practitioner with over two decades of experience spanning public health, ESG and sustainability, gender equality, business integrity, and public–private partnerships. At UN Global Compact Network India — the first formally recognised local network of the UN Global Compact — he leads initiatives on corporate governance, health governance, environmental governance, anti-corruption, and the Sustainable Development Goals, and has steered the organisation's Centre of Excellence for Governance, Ethics and Transparency.

He has previously partnered with PHDCCI on corporate governance and ethical-leadership initiatives. His expertise anchors the Cross-Track Sustainability & Community categories, ensuring that environmental and community claims are evaluated against credible SDG-aligned standards rather than compliance-driven box-ticking.

United Nations Development Programme (UNDP)

Institutional Jury Partner · United Nations

The United Nations Development Programme (UNDP) lends institutional weight to the panel as a partner in the evaluation of development impact and sustainability. UNDP's involvement reinforces the programme's commitment to outcomes that align with global development goals — equitable access, environmental responsibility, and measurable community benefit — and signals to nominees that the Samman is benchmarked against international standards of credibility and impact.

Dr. Nischal Naveen

Founder & Chairman, Voice of Healthcare · Co-Founder, Meddo Health · Founder, Cygnus Hospitals

Dr. Nischal Naveen is one of India's foremost healthcare entrepreneurs and a nationally recognised voice in healthcare policy reform. He founded Voice of Healthcare (VOH) in 2016 — a policy think tank that actively advocates with the Government on healthcare policy, delivery frameworks, and system-level reform. Under his leadership, VOH has grown into a substantive platform that convenes clinicians, entrepreneurs, hospital leaders, investors, and policymakers, and in 2023 he extended this into the VOH Incubator to actively build the next generation of healthcare start-ups. He currently serves on the Advisory Committee of the IIM Jammu MBA in Healthcare Administration and Management.

Chief Guest: Shri Subodh Uniyal, Hon'ble Cabinet Minister, Government of Uttarakhand (written confirmation awaited), is invited to grace the 2026 edition — underscoring the partnership between industry and the state government in advancing the Himalayan healthcare agenda.

7. The Concept of Tracks, Categories, and Evaluation

The recognition framework is organised into tracks so that like is judged against like. Each track corresponds to a distinct part of the ecosystem with its own regulatory context, its own evidence base, and its own peer group. Within each track, every Samman is a single, answerable evaluation question. Each track also carries a distinct colour identity used consistently across the website, nomination forms, and the gala.

7.1 Programme architecture at a glance

Track	Count	Open to	Colour
A — Public Health	7	Government health facilities, departments, frontline workers	Teal
B — Private Healthcare	6	Private hospitals, clinics, diagnostic chains, start-ups	Blue
C — AYUSH	11	AYUSH practitioners, institutions, manufacturers, researchers	Amber
D — Healthcare Manufacturers	12	Pharma, AYUSH, and medical-device manufacturers	Purple
E — Nutraceuticals (new)	8	FSSAI-licensed nutraceutical manufacturers	Pink
Cross-Track — Sustainability & Community	5	All participants across every track (public / private peer groups)	Green
Distinguished Achievers' Citations	10	By jury nomination / recommendation only — not direct application	Coral
TOTAL	59		

Two structural innovations underpin this architecture. First, the **Cross-Track section** consolidates sustainability and community recognition into one place, open to every track, with public-sector and private-sector nominees judged in separate peer groups so a government PHC is never measured against a large corporate hospital. Second, **size-banding** in Tracks D and E (Industry Leader above ₹500 Cr, Mid-Scale ₹100–500 Cr, Emerging up to ₹100 Cr) means scale determines the competitive pool automatically, removing ambiguity and protecting MSMEs.

7.2 The competitive tracks and their categories

Track A — Public Health (Teal)

Open to government health facilities, departments, PPP entities, and frontline health workers. Track A categories are not open for paid subscription: government facilities and frontline workers are assessed through established government quality frameworks — Kayakalp and NQAS — together with the recommendation framework of the State Department of Health & Family Welfare and demonstrated quality, reach, and outcomes. No nomination fee applies.

Code	Samman
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A1	Innovation in Public Health Samman
A2	District Hospital Samman
A3	CHC / Sub-Divisional Hospital Samman
A4	Primary Health Centre Samman
A5	Ayushman Arogya Mandir Samman
A6	Excellence in Public Health Infrastructure Samman
A7	Frontline Champion Samman — ANM / ASHA Worker

Track B — Private Healthcare (Blue)

Open to private hospitals, clinics, diagnostic chains, and healthcare start-ups. Looks for performance beyond regulation — clinical breakthroughs, technology adoption, patient experience, and employer excellence.

Code	Samman
B1	Technology and Innovation Leader Samman
B2	Startup of the Year Samman
B3	Clinical Excellence Leader Samman
B4	Patient Experience Leader Samman
B5	Healthcare Employer of the Year Samman
B6	Medical Value Travel Leader Samman

Track C — AYUSH (Amber)

The widest track, reflecting the region's AYUSH depth — from individual practitioners and therapists to institutions, researchers, manufacturers, and exporters. Predominantly individual and small-enterprise pricing.

Code	Samman
C1	Wellness Excellence Samman
C2	Distinguished Yoga Practitioner Samman
C3	Distinguished Panchakarma Practitioner Samman
C4	AYUSH Clinical Excellence Samman — Government Service
C5	AYUSH Clinical Excellence Samman — Private Practice
C6	AYUSH Centre of Excellence Samman
C7	AYUSH Innovation and Research Samman
C8	AYUSH Startup of the Year Samman
C9	AYUSH Pharmaceutical Excellence Samman
C10	AYUSH Entrepreneurial Achievement Samman

C11 AYUSH Export Excellence Samman**Track D — Healthcare Manufacturers (Purple)**

Open to pharma, AYUSH, and medical-device manufacturers. Business-excellence categories are size-banded so emerging enterprises compete with peers, not industry leaders. A clean compliance record is a precondition.

Code	Samman
D1a	Outstanding Business Growth Samman — Industry Leader (above ₹500 Cr)
D1b	Outstanding Business Growth Samman — Mid-Scale (₹100–500 Cr)
D1c	Outstanding Business Growth Samman — Emerging (up to ₹100 Cr)
D2a	Excellence in Global Representation Samman — Industry Leader
D2b	Excellence in Global Representation Samman — Mid-Scale
D2c	Excellence in Global Representation Samman — Emerging
D3	Commitment to Quality and Compliance Samman
D4	Breakout Smart Packaging Samman
D5	In-House Laboratory Excellence Samman
D6	Breakout R&D and Innovation Samman
D7	Employer and Workforce Excellence Samman
D8	Technical Workforce Excellence Samman

Track E — Nutraceuticals (Pink) · New for 2026

A dedicated track for FSSAI-licensed nutraceutical manufacturers — the fastest-growing segment of the wellness economy. White-label and third-party manufacturers are explicitly eligible; brand ownership is not required. A mandatory label declaration (“NOT FOR MEDICINAL USE”) applies, and products claiming to cure disease are ineligible.

Code	Samman
E1a	Outstanding Business Growth Samman — Industry Leader (above ₹500 Cr)
E1b	Outstanding Business Growth Samman — Mid-Scale (₹100–500 Cr)
E1c	Outstanding Business Growth Samman — Emerging (up to ₹100 Cr)
E2a	Excellence in Global Reach Samman — Industry Leader
E2b	Excellence in Global Reach Samman — Mid-Scale

E2c	Excellence in Global Reach Samman — Emerging
E3	Commitment to Quality and Compliance Samman — Nutraceuticals
E4	Breakout Product Innovation Samman — Nutraceuticals

Cross-Track — Sustainability & Community (Green)

The structural innovation of 2026: open to participants from every track, with public-sector and private-sector nominees evaluated in separate peer groups so sustainability and community impact are judged once, consistently, against SDG-aligned standards.

Code	Samman
CT1	Environmental Sustainability Leader Samman — Public Sector
CT2	Environmental Sustainability Leader Samman — Private Sector
CT3	Community Health Outreach Leader Samman — Public Sector
CT4	Community Health Outreach Leader Samman — Private Sector
CT5	Technology-Driven and CSR Impact Champion Samman

Distinguished Achievers' Citations (Coral) · By jury prerogative only

Conferred purely at the prerogative of the jury, these recognitions are not open for subscription and carry no nomination fee. Institutional citations are drawn from the pool of nominees with two or more competitive nominations in the relevant track. Lifetime Achievement and Pioneer citations are by Letter of Recommendation and Detailed Profile submitted to PHDCCI by a recognised institution or industry body. There is no self-nomination and no application form for any citation in this section.

Code	Samman
J1	Private Medical Institution of the Year
J2	Private Medical College of the Year
J3	AYUSH Institution of the Year
J4	AYUSH Medical College of the Year
J5	Nutraceutical Company of the Year
J6	Lifetime Achievement — Doctor in Govt Service
J7	Lifetime Achievement — Doctor in Private Practice
J8	Lifetime Achievement — AYUSH Doctor (Govt Service)
J9	Lifetime Achievement — AYUSH Doctor (Private Practice)
J10	Pioneer of Nutraceuticals

7.3 Documentary requirements

Every category card on the website displays its documentary requirements before a nominee applies, so the right category and the evidence needed are clear up front. This is intentional and confirmed practice. Requirements are organised in three layers, kept deliberately short so they fit on a category card without scrolling.

Layer 1 — Universal mandatory documents (identical across all categories)

- **GST returns of the preceding two quarters** — to establish that the nominee is a functioning unit as on the date of evaluation. (Government institutions substitute an appointment/sanction letter; individual practitioners substitute council registration.)
- **A valid licence relevant to the category** — the operating or manufacturing licence appropriate to the track (see the track-licence rule below).
- **Key team-member details** — mandatory, so that the team can be invited on stage on the gala night to receive the recognition on behalf of the nominee.

Layer 2 — Category-specific mandatory documents

Up to four documents specific to each Samman — the evidence that lets the jury answer that category's evaluation question (for example, clinical-audit data for B3, published research for C7, or export documentation for D2/E2). Four is the deliberate ceiling: rigorous enough to judge, short enough to display.

Layer 3 — Optional supporting documents

Two or three optional items — media coverage, testimonials and feedback, and additional certifications. These strengthen a nomination and differentiate borderline cases at jury level, but their absence does not gate eligibility.

The track-licence rule. The licence field changes by track, and this is stated on each track's cards so there is no ambiguity for nominee or web team:

Track	Relevant licence / credential
A — Public Health	Government appointment / sanction letter for the facility or post
B — Private Healthcare	Clinical Establishment Registration Certificate
C — AYUSH	AYUSH manufacturing licence or relevant council registration
D — Manufacturers	CDSCO / State Drug Licensing Authority manufacturing licence
E — Nutraceuticals	FSSAI Central License (verifiable on FoSCoS)

Disqualification safeguards. Two tracks carry an explicit, declared safeguard backed by a signed declaration on company letterhead, not a mere assertion: in Track D, any drug-sample-failure order — domestic or international — in the preceding 24 months bars the manufacturer; in Track E, any FSSAI enforcement action or product recall bars the manufacturer, and any product without the “NOT FOR MEDICINAL USE” declaration is ineligible.

7.4 What the jury evaluates

Eligibility (the universal documents and the track licence) is settled before a nomination ever reaches the jury — an incomplete nomination simply does not get forwarded. The jury's attention is therefore reserved entirely for excellence. Each nomination is scored independently against a structured rubric of four to six criteria, weighted to the category. The recurring evaluation dimensions are:

- **Performance beyond regulation** — evidence of going measurably past the statutory minimum: new clinical pathways, new processes, new products, new service models.
- **Outcomes and hard data** — verifiable metrics, benchmarks, and results, directly addressing the 2025 jury's observation that submissions too often lacked hard data.¹²
- **Innovation and research** — patents, publications, regulatory approvals, technology adoption, and demonstrable R&D output.
- **Quality and compliance depth** — accreditations, in-house testing, documentation discipline, and a clean regulatory record.
- **Community and sustainability impact** — measurable community health outcomes and SDG-aligned environmental practice, evaluated against credible standards rather than compliance claims.
- **Reach, scale, and global representation** — documented exports, international accreditation, and demonstrated growth, judged within the appropriate size-band peer group.

The process is designed to respect the jury's time and protect its independence: nominations are compiled into a standardised dossier per category, scored independently in advance on a digital rubric, auto-aggregated, and only borderline or divergent cases are discussed in a final meeting. The jury evaluates excellence; the framework handles eligibility. Optional supporting material — media, testimonials, certifications — is used precisely to separate closely-matched nominees.

¹²

8. Conclusion and Way Forward

India's healthcare and AYUSH economy is expanding faster than almost any sector of the national economy, and Uttarakhand sits at a uniquely advantageous intersection of clinical care, traditional healing, and life-sciences manufacturing. What the region lacks is not capability but **recognition, evidence, and reputation** — precisely the gap the 2025 jury identified and precisely the gap this Samman is built to close.

The 2026 edition answers that need with discipline. A sprawling, reactive list has been consolidated into 59 clearly-defined categories across five tracks, a Cross-Track section, and a Distinguished Achievers' Citations tier — each anchored to a question an expert jury can answer with evidence. Size-banding and peer-grouping make the competition fair. A uniform, transparent documentary standard, published before application and operated fee-first, makes the process orderly and credible. And a jury drawn from drug regulation, science, academia, and global sustainability governance makes the recognition mean something.

The Event happening on 24th June 2026 in Dehradun, the Uttarakhand Pharma Healthcare & AYUSH Conclave and Samman 2026 aspires to be more than an awards evening. It aims to set a standard for how excellence in Himalayan healthcare is documented, benchmarked, and celebrated — a discovery of excellence that goes beyond compliance, in service of a healthier region and a more visible, more competitive, more confident sector.

PHD Chamber of Commerce and Industry · Healthcare, Education & Research Committee
Uttarakhand Pharma Healthcare & AYUSH Conclave and Samman 2026 · Dehradun